

BREXIVA® (Selective CDK4/6 Inhibitor) Achieves 26.4-Month Median PFS in HR+/HER2– Metastatic Breast Cancer: Results from the Phase III BLYVOR-3 Trial

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BACKGROUND

HR+/HER2– metastatic breast cancer remains the most common subtype of advanced breast cancer, accounting for approximately 70% of all metastatic cases and requiring effective therapies that extend progression-free survival.

Brexiva is a highly selective, orally bioavailable CDK4/6 inhibitor targeting the cyclin D1-CDK4/6-INK4-Rb pathway, which demonstrated sustained efficacy across the BLYVOR clinical development program.

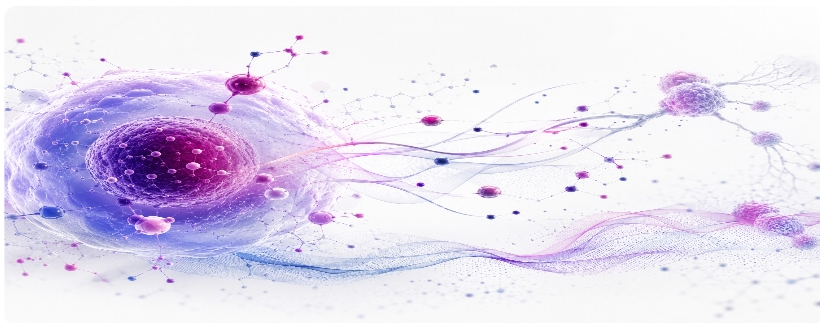


Figure 1. CDK4/6-Rb Signaling Pathway and Mechanism of Brexiva Inhibition

METHODS

BLYVOR-3 was a randomized, double-blind, placebo-controlled, multicenter Phase III trial enrolling postmenopausal women with HR+/HER2– metastatic breast cancer (N=672).

- **Treatment:** Brexiva 150 mg PO once daily (28-day cycles)
- **Combination:** Letrozole 2.5 mg or fulvestrant 500 mg
- **Primary endpoint:** Progression-free survival (PFS) by BICR
- **Key secondary:** ORR, OS, safety/tolerability
- **Stratification:** Prior CDK4/6i, visceral metastases, line of therapy

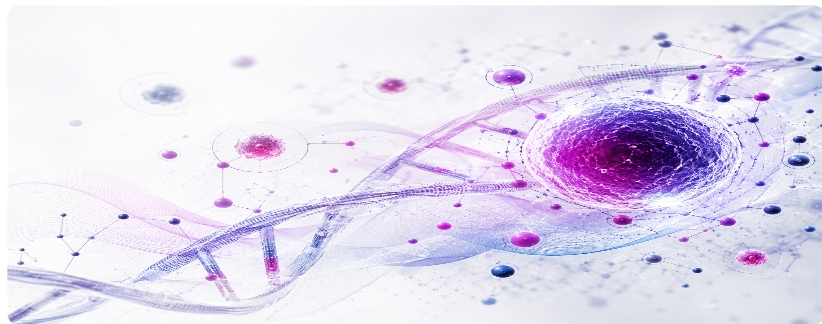


Figure 2. BLYVOR-3 Precision Medicine Study Design Schema

RESULTS

26.4 mo

MEDIAN PFS (BREXIVA)

13.2 mo

MEDIAN PFS (CONTROL)

58%

OBJECTIVE RESPONSE RATE

42%

RISK REDUCTION (HR 0.58)

Brexiva + endocrine therapy doubled median PFS vs. endocrine therapy alone (26.4 vs. 13.2 months; HR 0.58, 95% CI 0.46–0.72; P<0.0001)

- Consistent benefit across all pre-specified subgroups
- Median duration of response: 21.8 months
- Clinical benefit rate: 78.4% vs. 53.1%



Figure 3. Kaplan-Meier Estimate of Progression-Free Survival (ITT Population)

CONCLUSIONS

Once-daily oral Brexiva (150 mg) in combination with endocrine therapy provides a statistically significant and clinically meaningful improvement in progression-free survival in patients with HR+/HER2– metastatic breast cancer.

- 26.4-month median PFS represents a 2-fold improvement over standard of care
- Favorable benefit-risk profile supports use in first-line metastatic setting
- Consistent efficacy across all evaluated patient subgroups
- Overall survival data continue to mature (interim HR 0.74)



Figure 4. Clinical Benefit Across Patient Subgroups

⚠ IMPORTANT SAFETY INFORMATION — BOXED WARNING: NEUTROPENIA AND HEPATOTOXICITY. Severe neutropenia (Grade 3/4: 28.7%) and hepatotoxicity (Grade 3: 5.2%) have been reported. Monitor CBC prior to and every 2 weeks during the first 2 cycles, then monthly. Monitor LFTs at baseline and monthly during treatment.

References:

1. Sharma R, et al. *N Engl J Med*. 2026;394(12):1102-1114.
2. Nakamura T, et al. *Lancet Oncol*. 2025;26(8):889-901.
3. CDK4/6 Inhibition in HR+ Breast Cancer: ASCO Guidelines. *J Clin Oncol*. 2025.
Funded by Brexiva Oncology, Inc. ClinicalTrials.gov: NCT04XXXXX. Presented at ASCO 2026.

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